The Mental Health Coverage Gap in Indian Health Insurance:

Diagnosing the Gap, and Where It Closes *

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Eight years after the Mental Healthcare Act, 2017 mandated insurance parity for mental illness, and three years after IRDAI's compliance deadline of October 2022, the average urban Indian seeking outpatient therapy still pays cash. The gap is structural, not regulatory. Three mechanisms have neutralised the mandate: product formats built for inpatient hospitalisation only, provider eligibility rules that exclude most practising therapists, and outpatient unit economics that no insurer can underwrite at scale. This report works through each mechanism, quantifies the cost the therapy seeker absorbs, and identifies two intermediary business possibilities positioned to close the gap that retail insurance will not.

1 The Gap

Despite growing recognition of mental health as a public concern, and government efforts at destigmatisation, accessing care still depends largely on what a patient can pay out of pocket. IRDAI in 2022 mandated insurance coverage for mental illness as part of every retail health policy and three years on, though the policies technically comply, a customer seeking weekly therapy for anxiety or stress finds little of it actually reimbursable. A 2024 audit of 235 retail policies across 30 IRDAI-registered insurers? confirms the pattern: 51% of policies offer no mental illness coverage at all, only 11 of the 30 insurers provide any outpatient mental health coverage. Where coverage exists, it is largely restricted to premium-tier products. Where mental illness is covered, waiting periods of 24-48 months apply for mental illness, against a 30-day general waiting period for most other conditions. NMHS 2016 found common mental disorders like anxiety, depression, stress-related conditions, affect roughly 10% of Indians, against 0.8% prevalence for severe mental disorders. Insurance products are calibrated to the latter.

Policy	OPD therapy	MI inpatient	Waiting period	Provider eligibility	Suicide/Self-harm	Substance abuse
Future Generali Health Total	×	✓	24m	!*	×	×
SBI Arogya Premier	× !	✓ !	24–48m	!*	×	×

(1) For SBI Arogya Premier, OPD is available only as a paid add-on rider and is capped; mental illness inpatient cover is sub-limited to ₹1,00,000 against a base sum insured of ₹10–30 lakh.

(2) *Registered medical practitioner only; clinical psychologists and counsellors not recognised for reimbursement.

Even the two retail health products academically endorsed as offering most comprehensive mental health coverage (Avula et al., 2024) reimburse realistically close to 0% of any therapy course. Psychiatric treatment is excluded from Future Generali's OPD, and SBI Arogya Premier offers no OPD in its base policy. Ditto Insurance, advisor to over 800K retail customers, has acknowledged that most mental health claims they handle are denied because hospitalisation cannot be medically justified.

2 Why It Persists

Even after IRDAI's 2022 hard deadline, insurers' response was the predictable minimum compliance: technical inclusion, with expanded terms and limitations (waiting periods, provider eligibility, sub-limits, OPD exclusions). While the MHCA explicitly recognizes clinical psychologists licensed by RCI as legitimate providers of psychotherapy, insurance providers aren't obligated to include them in the pool of MBBS-qualified medical doctors registered with the NMC. If they can restrict hospitals, then why not care providers?

Even if this failure was fixed, OPD psychotherapy at scale doesn't underwrite profitably with indemnity-hospitalisation insurance. Hospitalisation insurance works because most insured customers never claim. Therapy doesn't work that way. Anyone who buys a therapy rider intends to use it weekly. For the

* Independent analysis; views are personal.

unit economics to work, the rider premium has to roughly equal expected payout. At that point, the customer might as well pay cash. Insurers comply with the mandate by extending hospitalisation coverage to psychiatric admission and offering capped OPD riders (4–6 visits per year, ₹500–1K per visit). They cannot offer OPD parity at a price point that would close the gap. The mandate made non-coverage illegal; it cannot make coverage profitable. Closing the gap requires market players operating outside retail-insurance economics.

3 Closing the Gap

A therapy seeker has no clear path to meaningful coverage. Retail insurance won't cover them, and employer-channel benefits, if exists, runs out before clinical efficacy is reached. A benefits platform (Loop, Plum, etc.) sit between the employer and the underlying insurer and currently offer minimal mental health support, typically an amorphous allotment of sessions with a D2C therapy partner (Amaha, Elfina etc.), varying by employer contract. A more useful design and a better service solution would commit to a fixed baseline of 8–10 employer-funded sessions, with continuation available to the employee at a negotiated rate (modestly below retail) paid directly to the therapy platform. The therapist remains fully paid; the customer with continued need keeps the same provider at meaningfully below retail; the platform passes the customer onto the therapy partner who continues to earn thin margin over high volume of customers at no acquisition cost, and preserves its customer relationship. The structure works because a group plan eases the underwriting risk significantly and the collective workforce subsidises service for the heavy users. The deeper partnership also addresses post-employment continuity: when an employee leaves the employer, they lose access to Loop's negotiated rate but keep their therapist relationship; the platform-side data and trust persist, and the customer continues with the same provider at retail pricing rather than starting over with someone new.

Insurance advisors could also address the gap by building a clause level depth in aspects like definition, OPD, waiting periods and exclusions, and offering guidance on how one can combine retail/employer cover with self-pay rationally. This helps the customer navigate the insurance market and reinforces the honest-broker positioning advisors already trade on.

The mandate made non-coverage illegal but not profitable; the gap will partly close through such initiatives of better benefit design in the B2B channel, and honest advisory on the retail side.

References

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